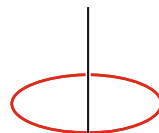


Initiating Coverage



DALAL & BROACHA
STOCK BROKING PVT. LTD.



KIMS - King in making

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Investment Argument:

In a populous country like India occupancy is not a constraint but affordability is. We initiate coverage on KIMS which is one of the lowest cost healthcare service providers with industry leading margins backed by consistent volumes driven by doctors who own equity in the company. We initiate coverage on KIMS with "BUY ON DIPS" rating for a target price of Rs.1,561 valuing company at 20x EV/EBITDA of FY24E Ex-sunshine & 15x EV/EBITDA of FY24E for sunshine

➤ Robust operating profit above 30% & cashflows to drive growth

- KIMS' generates industry leading EBITDA margins of above 30%. We expect KIMS to generate operating cashflows in the range of ~3,500 Mn – 4,000 Mn p.a. driven by higher occupancies & higher ARPOB (Average revenue per occupied bed) contribution for next 3-4 years. EBITDA to OCF conversion is above 80% which will support growth plans going forward.

➤ Bed capacity to expand ~50% by FY26

- Robust operating profit conversion will be used to increase bed capacity by ~50% by FY26 from current levels of 3,666 beds (including recent sunshine acquisition) to 5,391 beds mainly funded through internal accruals & short term debt to manage timing difference.

➤ Differentiated business model backed by affordable pricing & doctor participation model

- KIMS has unique business model which focuses on low cost affordable pricing & hence KIMS has one of the lowest ARPOBs & ARPPs (Average revenue per patient) but still generates industry leading margins of 30%+ (Refer exhibit 8 & 10)
- Out of total panel of 1,140 doctors nearly 300 doctors own ~9% equity in the company which drives volume for the company.

Financial Summary

Y/E Mar (Rs mn)	FY20	FY21	FY22E	FY23E	FY24E
Net sales	11,226	13,299	16,718	19,938	23,259
EBIDTA	2,450	3,709	5,270	5,839	6,727
Margins	21.8	27.9	31.5	29.3	28.9
PAT (adj)	1,192	2,012	3,405	3,591	4,101
Growth (%)	-	68.7	69.3	5.5	14.2
EPS	16.0	25.9	42.6	44.9	51.3
P/E (x)	83.1	51.3	31.2	29.6	25.9
P/B (x)	16.6	11.9	8.8	6.8	5.4
EV/EBITDA (x)	41.4	27.7	19.3	17.2	15.5
RoE (%)	19.9	23.3	28.2	22.9	20.8
ROCE (%)	19.9	27.3	36.4	30.8	27.8

Source: Dalal and Broacha

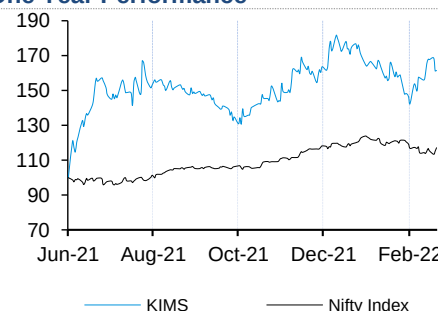
Rating	TP (Rs)	Up/Dn (%)
BUY ON DIPS	1,561	17

Market data

Current price	Rs	1,330
Market Cap (Rs.Bn)	(Rs Bn)	107
Market Cap (US\$ Mn)	(US\$ Mn)	1,392
Face Value	Rs	10
52 Weeks High/Low	Rs	1565 / 937
Average Daily Volume	('000)	363
BSE Code		543308
Bloomberg		KIMS.IN

Source: Bloomberg

One Year Performance



Source: Bloomberg

% Shareholding	Dec-21	Sep-21
Promoters	38.84	38.84
Public	61.16	61.16
Total	100	100

Source: Bloomberg

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➤ **Revenues to double to ~Rs.35 Bn. by FY26 through organic & inorganic expansion**

- KIMS has grown from single hospital in year 2000 to 12 hospitals in 2022(including recent sunshine acquisition) through greenfield projects & acquisitions.KIMS has strong regional presence in southern India & scale leadership with bed capacity at ~1.9x that of second largest player in AP & Telangana.
- They have significantly expanded their hospital network in recent years through their acquisitions of hospitals in Ongole in FY17, Vizag and Anantapur in FY19,Kurnool in FY20 & Sunshine in FY22 which added 1,836 beds to overall capacity.
- Approximately one-third of the total 3,064 beds(excluding sunshine) were added in the last four years and improved the overall bed occupancy rate in these hospitals from 71.83% to 78.6% in the same period.
- KIMS has delivered healthy revenue/EBITDA/PAT growth of 21%/28%/49% between FY16-21. Going forward we expect KIMS to deliver revenue/EBITDA/PAT growth of 20%/19%/17% between FY22-26.

➤ **Capex plan of ~Rs.14 Bn**

(FY21 Gross block : Rs.10 Bn)

- KIMS has planned major capex of ~Rs.14,000 Mn across tier1/2/3 cities across geographies (Kondapur, vizag, anantapur, ongole, banglore, chennai, western & central india) & is expected to increase its overall capacity by ~50% by FY26 from current levels of 3,666 beds (including recent sunshine acquisition) to 5,391 beds at average capex of Rs.8.14 Mn per bed.
- KIMS capex plan is supported by robust EBITDA conversion to OCF of >80% in the range of ~3,500-4,000 Mn. Company plans not to exceed debt levels beyond 1.5-2x of EBITDA.

➤ **Dependency of key hospital to reduce substantially going forward**

- We expect KIMS contribution from flagship facility at Secunderabad to go down from ~52%/63% of overall revenue/EBITDA in FY21 to 29%/35% in FY26 due to new greenfield projects & sunshine acquisition, thereby deleveraging its overall risk but sustaining EBITDA margins in the range of 28-30%.

➤ **Focus on affordable healthcare to drive occupancy**

- From the very beginning KIMS has focused on affordable healthcare due to cost rationalization which is the need of the hour.In populous country like India occupancy is not a constraint,but affordability is. Occupancy is largely driven by

affordability. KIMS had one of the lowest ARPOBs of Rs.20,609 & ARPP of Rs.1,13,968 in FY21 compared to peers which makes it affordable to large masses. *(Refer exhibit 8 & 10)*

➤ **Scope for margin improvement**

- Recent acquisitions at Vizag (FY19), Anantapur (FY19), Kurnool (FY20), Sunshine (FY22) contributes 17%/9%/8%/18% EBITDA margin respectively which is significantly lower than the company level margin of 30%+. These facilities contribute ~6,000 Mn revenue in FY21 & there is scope of margin improvement as they reach their mature phase. Process of margin improvement is explained in *exhibit 32*

➤ **High ARPOB generating facilities to aid further growth**

- KIMS company level ARPOB to reach ~Rs.29,000 by FY26 from current levels of ~Rs.24,500 in FY22 on account of expansion in metro cities like Bangalore, Chennai & Mumbai which are high ARPOB cities.

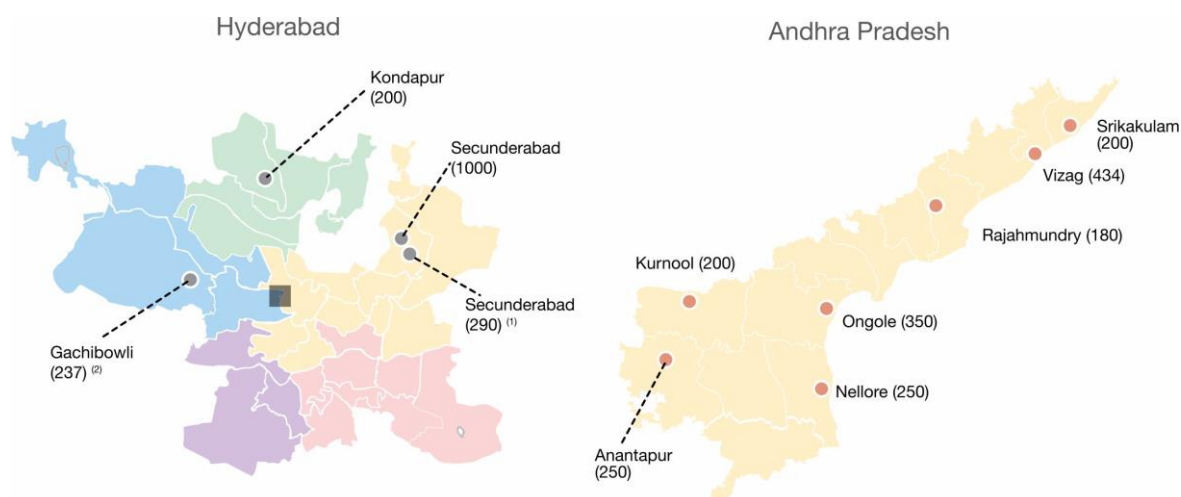
Exhibit 1: Network of 12 hospitals

KIMS has a dominant presence in South India region with a chain of 12 hospitals including recent Sunshine Acquisition. Their flagship hospital at Secunderabad is one of the largest private hospitals in India at a single location with a capacity of 1,000 beds. KIMS has focused on underpenetrated Tier 2 & Tier 3 cities where it has 53% of total commissioned beds.

KIMS Hospital Ownership, Bed Capacity & Property Ownership

Particulars	Ownership (%)	Bed Capacity	Operational Beds	Property Owned or Lease	Tier I - II - III
Secunderabad	100	1,000	885	Perpetual lease	Tier I
Nellore	100	250	250	Part owned & part leased (30 yrs)	Tier II-III
Rajamundry	100	180	180	Part owned & part leased	Tier II-III
Srikakulam	58	200	150	Owned	Tier II-III
Kondapur	86	200	150	Leased (10 yrs)	Tier I
Ongole	100	350	246	Owned	Tier II-III
Vizag	51	434	314	Service Agreement	Tier II-III
Anantpur	80	250	215	Owned	Tier II-III
Kurnool	55	200	200	Owned	Tier II-III
Sunshine Hospitals (Secunderabad)	51	290	192	Leased	Tier I
Sunshine Hospitals (Gachibowli)	51	237	158	Leased	Tier I
Sunshine Hospitals (Karimnagar)	51	75	50	Leased	Tier II-III
Total		3,666	2,990		

Source : Company, Dalal & Broacha research, Sunshine operational beds on proportionate basis



Source : Company, Dalal & Broacha research, 1 & 2) Related to Sunshine Hospital

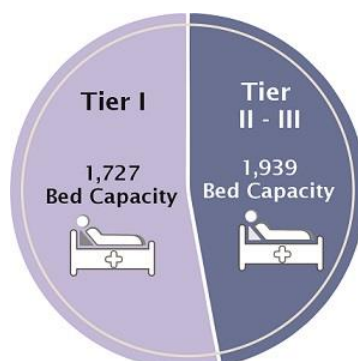


Exhibit 2: KIMS Operating metrics across hospitals

Particulars	Secbad.	Nellore	Rajmudry	Srikakulam	Kondapur	Ongole	Vizag	Anantapur	Kurnool	Sunshine Hospitals
Year of commencement	2004	2000	2002	2011	2014	2016	2018	2018	2019	2022
Bed Capacity	1,000	250	180	200	200	350	434	250	200	602
Operational beds	885	250	180	150	150	246	314	215	200	400
Bed occupancy rate on operational beds (%) *	66	89	83	64	82	97	89	98	64	N.A.
Bed occupancy rate on commissioned beds (%)	49	76	76	43	49	64	44	70	58	N.A.
ALOS (days)	4.97	5.13	4.76	4.87	3.89	8.21	6.16	6.02	4.58	5.0
ARPOB (₹ per day)	38,478	13,563	15,035	11,445	44,965	8,915	11,897	10,785	12,380	38,447
Inpatient volume	35,818	13,571	10,477	6,455	9,269	9,925	11,238	10,654	9,185	21,640
Outpatient volume	253,174	124,126	47,265	37,787	78,535	88,400	85,803	58,298	56,823	N.A.
Hospital revenue (₹ mn)	6,845	944	750	360	1,620	727	824	692	521	4,160
Revenue from inpatients (₹ mn)	5,759	602	475	238	1,323	483	661	527	412	N.A.
Revenue from outpatients (₹ mn)	1,086	342	274	121	296	244	162	165	108	N.A.
EBITDA (₹ mn)	2,433	278	196	53	419	230	139	60	39	752
EBITDA Margins FY21 (%)	35	29	26	15	26	32	17	9	7	18
EBITDA Margins FY20 (%)	28	25	23	18	25	16	-3	9	-3	9

Source : Company(FY21), Dalal & Broacha research

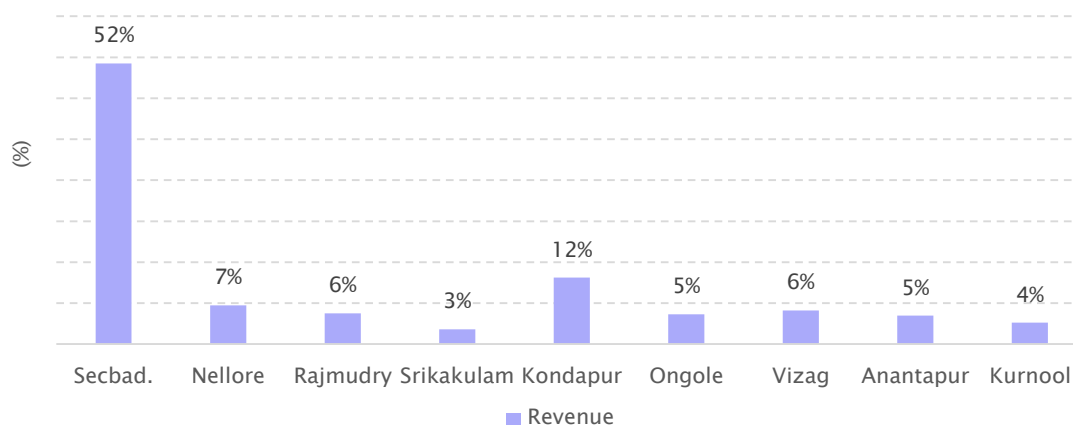
*Number of beds occupied divided by number of operational census beds

Recent acquisitions at Vizag(FY19),Anantapur(FY19),Kurnool(FY20),Sunshine(FY22) contributes 17%/9%/8%/18% EBITDA margin respectively which is significantly lower than the company level margin of 30%+.These facilities contributes ~6,000 Mn revenue in FY21 & there is scope of margin improvement as they reach their mature phase.Process of margin improvement is explained in **exhibit 32**.

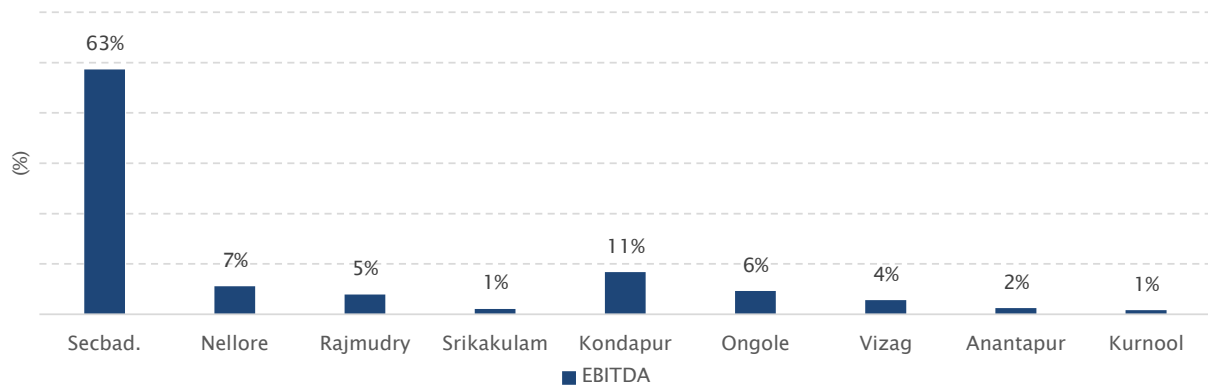
Complex heart & lung transplants are high EBITDA margin generating business which are introduced at flagship secunderabad facility which has led to margin improvement.

Exhibit 3: We expect KIMS revenue & EBITDA contribution from Secunderabad facility to fall drastically from 52%/63% in FY21 to 29%/35% in FY26 on account of greenfield & brownfield expansion which are high ARPOB generating facilities.

Hospitalwise Revenue distribution as a % of total revenue



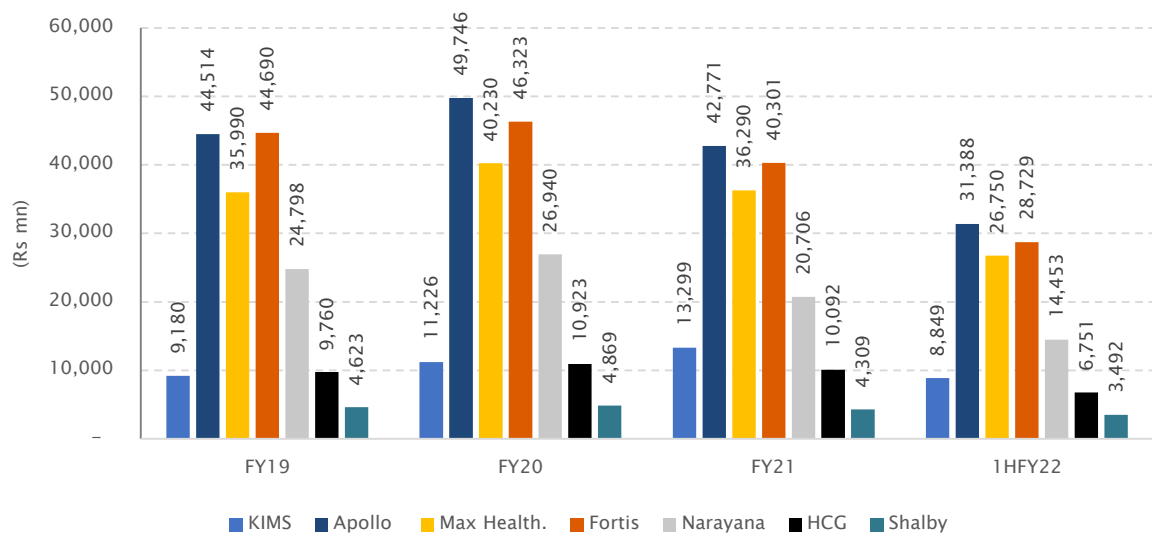
Hospitalwise EBITDA distribution as a % of total EBITDA



Source : Company(FY21 Figures), Dalal & Broacha research

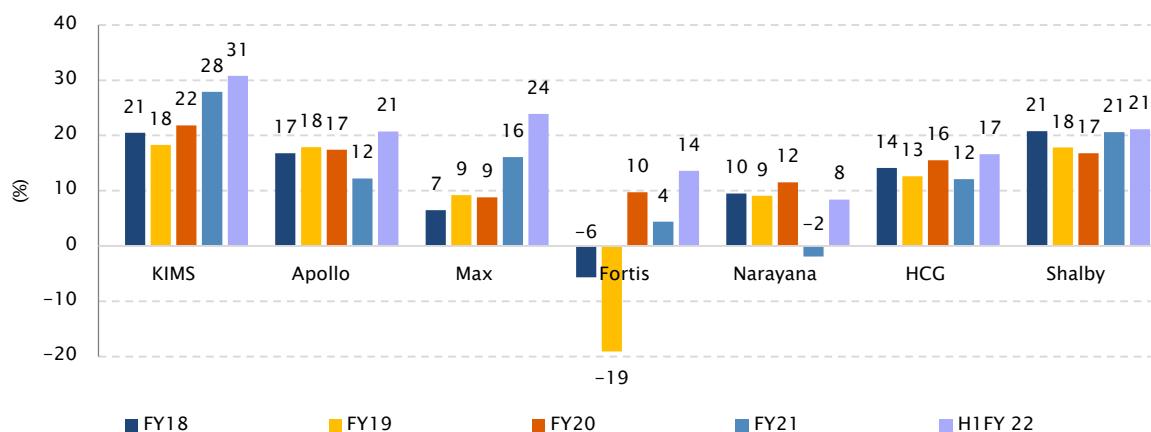
Exhibit 4: KIMS is the 5th largest player in terms of revenue across listed hospitals & steadily growing

Revenue comparison between peers

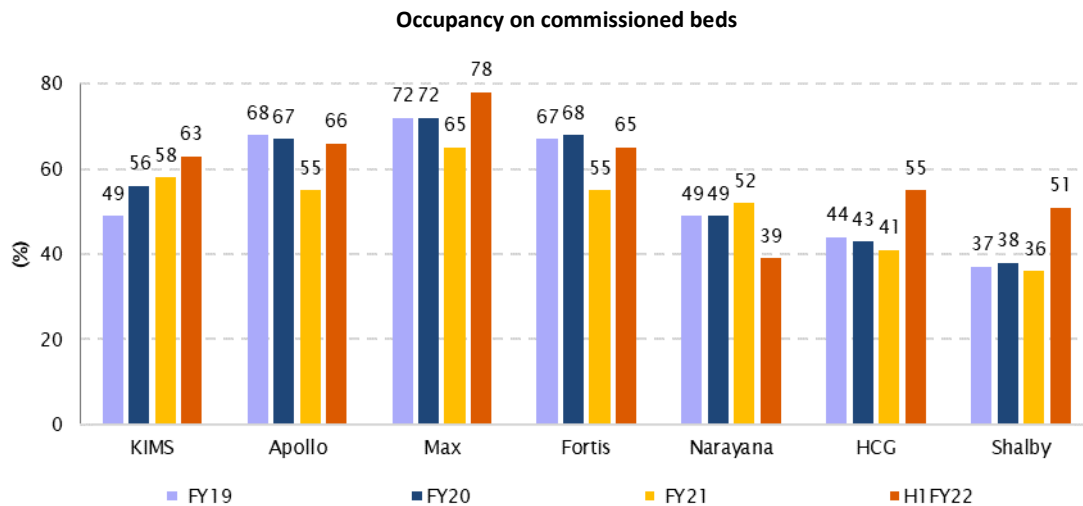


Source : Company, Dalal & Broacha research

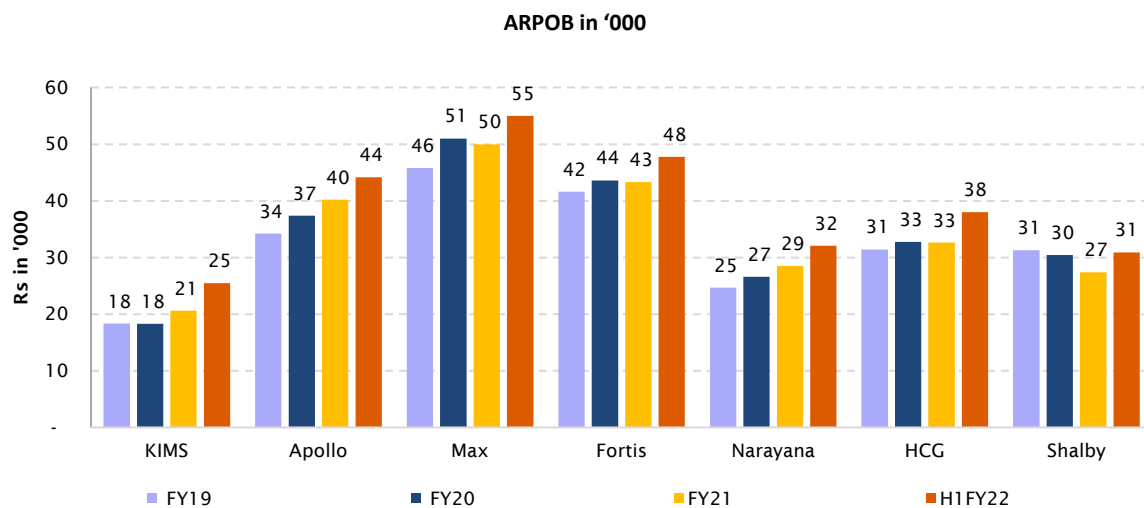
Exhibit 5: KIMS generates industry leading margin of 30%+ because of its focus on cost rationalisation & low capex per bed. KIMS focus on owning land in advance or procuring land on long term lease basis has helped cost savings of lease rentals.



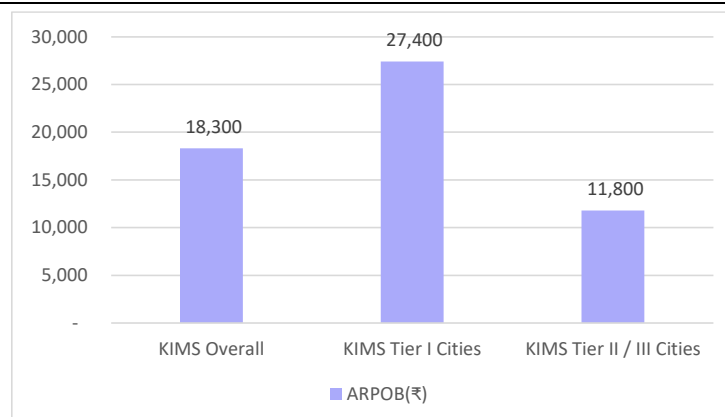
Source : Company, Dalal & Broacha research

Exhibit 6: Occupancy of KIMS has been continuously rising & is above ~50%

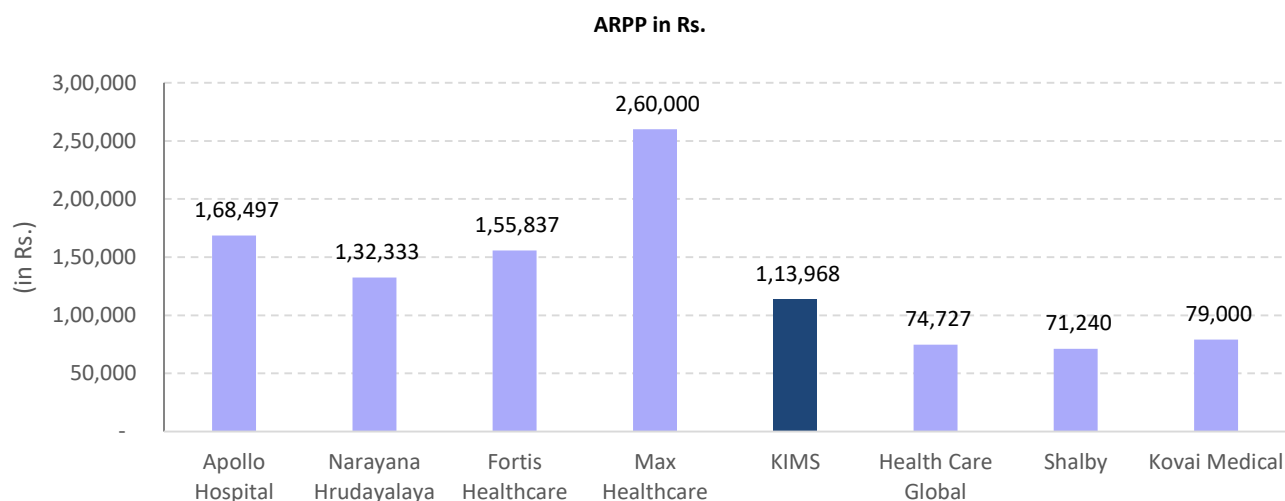
Source : Company, Dalal & Broacha research

Exhibit 7: KIMS ability to rationalise cost has made it an affordable yet quality health service provider. KIMS has one of the lowest ARPOB compared to peers however it is able to generate industry leading margin of 30%+ based on high volumes & high occupancies

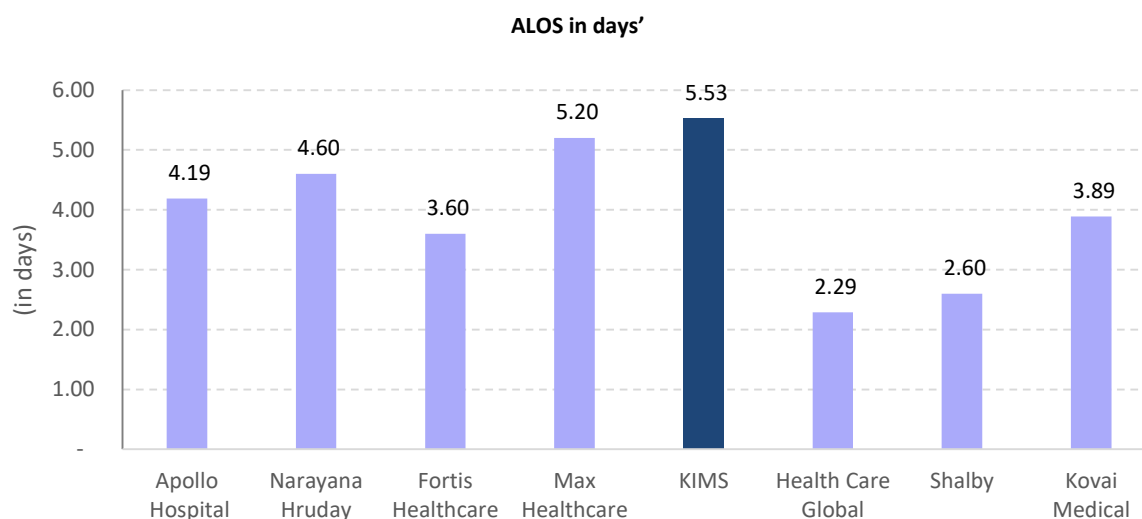
Source : Company, Dalal & Broacha research

Exhibit 8: ARPOB in Tier I & Tier II/III cities

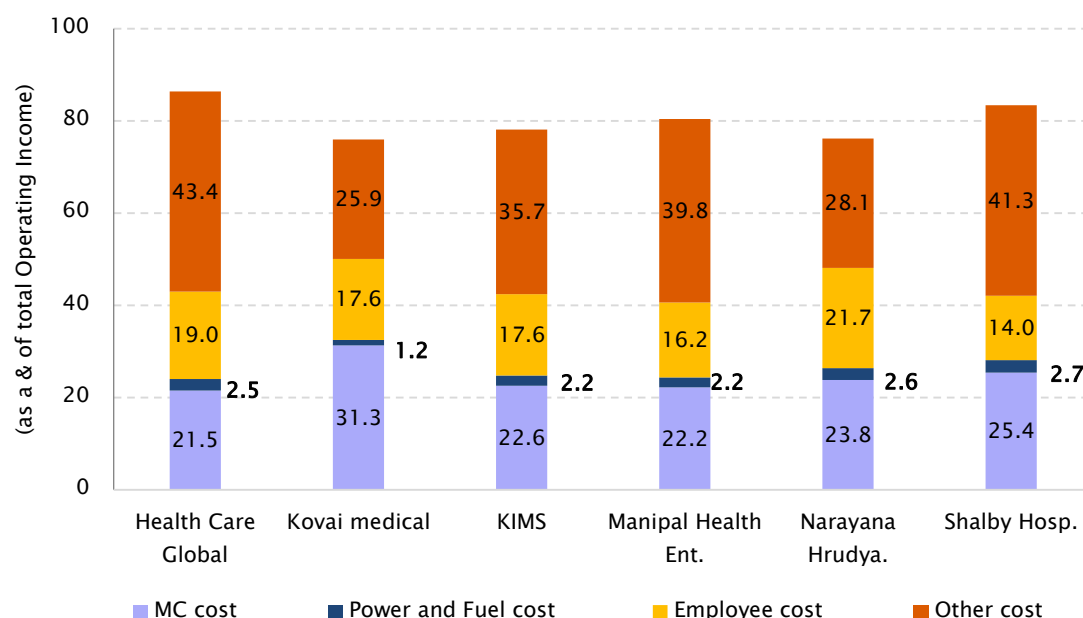
Source : Company(FY20), Dalal & Broacha research

Exhibit 9: KIMS has one of the lowest ARPP which makes it an affordable healthcare provider

Source : Company (FY21 figures), Dalal & Broacha research

Exhibit 10: ALOS of KIMS is slightly higher compared to peers at ~5.5 days because 30% volume is generated from state government schemes in the state of Andhra Pradesh which usually have higher length of stay

Source : Company(FY21 figures), Dalal & Broacha research

Exhibit 11: Cost Structure comparison between peers

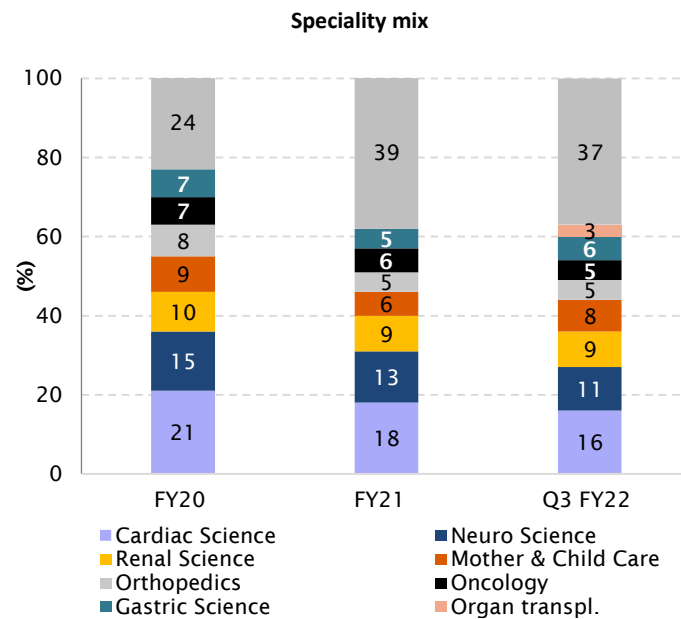
Source : Company RHP(FY20), Dalal & Broacha research

Exhibit 12: KIMS unique doctor participation model (300+ doctors own nearly ~9% equity in the company) drives volume & leads to higher occupancy, which also reduces attrition rate at the top level who are the key drivers of hospital business. Also KIMS has strong regional brand which makes it a preferred choice for patients.

Sr.no	Name of the Company	Name of Hospital	Shareholding % of Company	Shareholding % of Doctors
1	Krishna Institute of Medical Sciences	KIMS Secunderabad, KIMS Nellore, KIMS Rajahmundry and KIMS Ongole	100.00%	6.92%
2	KIMS Hospital Kurnool Private Limited	KIMS Kurnool	55.00%	45.00%
3	Iconkrishi Institute of Medical Sciences Private Limited	KIMS Vizag	51.00%	49.00%
4	Arunodaya Hospitals Private Limited	KIMS Srirakulam	57.83%	39.16%
5	Saveera Institute of Medical Science Private Limited	KIMS Anantapur	80.00%	20.00%
6	KIMS Hospital Enterprises Private Limited	KIMS Kondapur	86.32%	0.46%

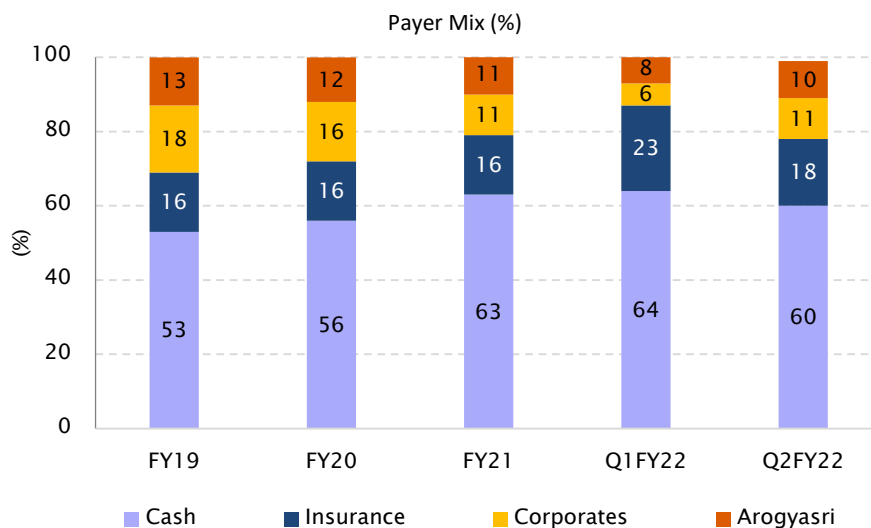
Source : Company, Dalal & Broacha research

Exhibit 13: KIMS offer comprehensive range of Services across 25+ Specialties including complex heart & lung transplants which were started a year back but expected to contribute ~700-800 Mn p.a. (4% of overall revenue).KIMS does not have dependency on any particular speciality thereby reducing overall risk.



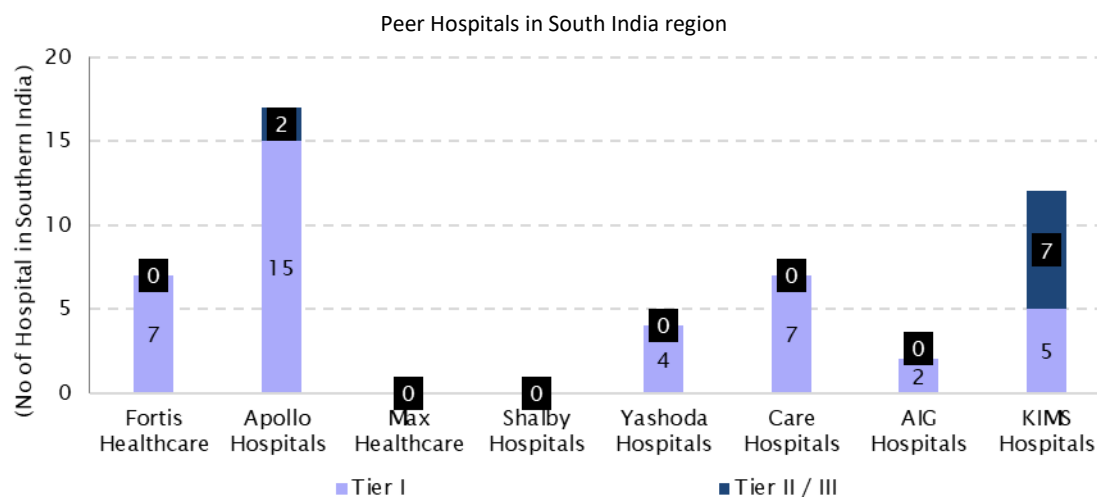
Source : Company, Dalal & Broacha research

Exhibit 14: KIMS corporate & aarogyasri payer mix fell from 31% in FY19 to 21% to Q2FY22 which are usually low margin contributors indicating better payer mix & margin profile



Source : Company, Dalal & Broacha research

Exhibit 15: KIMS has the highest tier II / III hospital mix in the southern india whereas other players have focused on tier I cities. Few large players like Max & Shalby are not present in southern india.



Source : Dalal & Broacha research

Competitors Name	No. of Hospitals	State	City	District,Road	Tier I/II - III city
Fortis Healthcare	7	Tamil Nadu	Chennai	Adyar	Tier I
		Tamil Nadu	Chennai	Vadapalani	Tier I
		Karnataka	Bengaluru	Bannerghata road	Tier I
		Karnataka	Bengaluru	Cunningham road	Tier I
		Karnataka	Bengaluru	Nagarbhavi	Tier I
		Karnataka	Bengaluru	Rajajinagar	Tier I
		Karnataka	Bengaluru	Richmond Town	Tier I
Apollo Hospitals	17	Tamil Nadu	Chennai	Apollo main hos pitals Greams Road	Tier I
		Tamil Nadu	Chennai	Apollo Specialiy hos pitals Nandanam	Tier I
		Tamil Nadu	Chennai	Apollo hos pitals Tondiarpet	Tier I
		Tamil Nadu	Chennai	Apollo First med hos pitals kilpauk	Tier I
		Tamil Nadu	Chennai	Apollo hos pitals Sowcarpet	Tier I
		Tamil Nadu	Chennai	Apollo heart centre Greams Road	Tier I
		Tamil Nadu	Chennai	Apollo childrens hos pitals thous and lights	Tier I
		Tamil Nadu	Chennai	Apollo Specialiy hos pitals vanagaram	Tier I
		Tamil Nadu	Chennai	Apollo medical centre Karapakkam	Tier I
		Tamil Nadu	Chennai	Apollo Speciality Hos pitals OMR	Tier I
		Andhra Pradesh	Chitoor	Apollo Hos pital Aragonda	Tier I
		Tamil Nadu	Madurai	Apollo speciality hos pitals KK nagar	Tier I
		Tamil Nadu	Madurai	Apollo First med hos pitals madurai	Tier I
		Tamil Nadu	Karur	Apollo Loga hos pitals kovai road	Tier I
		Tamil Nadu	Karaikudi	Apollo speciality hos pitals kallal panchayat union	Tier I
		Tamil Nadu	Trichy	Apollo speciality hos pitals chennai madurai bypas s road	Tier II
		Andhra Pradesh	Nellore	Apollo speciality hos pitals	Tier II
Max Healthcare		No Hospitals in south india			
Shalby Hospitals		No Hospitals in south india			
Yashoda Hospitals	3	Telangana	Secunderabad	Alexander Road, Secunderabad, Hyderabad	Tier I
		Telangana	Hyderabad (Somajiguda)	Rajbhavan Road, Somajiguda, Matha Nagar, Hyderabad, Telangana	Tier I
		Telangana	Malakpet	Nalgonda X Roads , Malakpet, Hyderabad	Tier I
(Upcoming)	1	Telangana	Hitech City (Indias largest 2000 beds)	Kothaguda, Hyderabad, Telangana	Tier I

Competitors Name	No. of Hospitals	State	City	District/Road	Tier I/II - III city
Care Hospitals	6	Telangana	Banjara hills (Hyd)	Care super speciality hospital and transplant centre	Tier I
		Telangana	Banjara hills (Hyd)	Care super speciality hospital OPD Centre	Tier I
		Telangana	Hitech City (Hyd)	Care super speciality hospital	Tier I
		Telangana	Nampally (Hyd)	Care super speciality hospital	Tier I
		Telangana	Musheerabad (Hyd)	Care super speciality hospital	Tier I
		Telangana	Bhubaneswar	Care super speciality hospital, chandrasekharapur	Tier I
AIG Hospitals	2	Telangana	Hyderabad (Gachibowli)	Mindspace Road, Gachibowli	Tier I
		Telangana	Hyderabad (Somajiguda)	Somajiguda, Sangeet Nagar	Tier I
KIMS Hospitals	12	Telangana	Secunderabad		Tier I
		Andhra Pradesh	Nellore		Tier II-III
		Andhra Pradesh	Rajahmundry		Tier II-III
		Andhra Pradesh	Srikakulam		Tier II-III
		Telangana	Kondapur		Tier I
		Andhra Pradesh	Ongole		Tier II-III
		Andhra Pradesh	Vizag		Tier II-III
		Andhra Pradesh	Anantapur		Tier II-III
		Andhra Pradesh	Kurnool		Tier II-III
		Telangana	Sunshine Secunderabad		Tier I
		Telangana	Sunshine Gachibowli		Tier I
		Telangana	Sunshine Karimnagar		Tier I

Source :Dalal & Broacha research

Exhibit 16: Regional Revenue Mix of key listed players

Particulars	North	West	East	South
Apollo Hospitals	-	-	41%	59%
Narayana Hrudayal	8%	15%	33%	44%
Fortis Healthcare	36%	29%	22%	13%
HEG	2%	44%	8%	46%
Kovai medical	-	-	-	100%
KIMS	-	-	-	100%

Source : Company RHP(FY20), Dalal & Broacha research

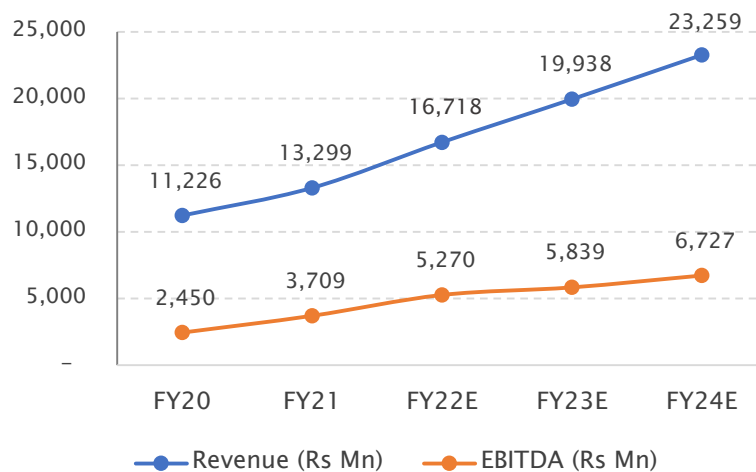
Exhibit 17: KIMS has planned major capex of ~Rs.14,000 Mn in tier1/2/3 cities across geographies (Kondapur,vizag,anantapur,ongole,bangalore,chennai,western & central india) & is expected to increase its overall capacity by ~50% by FY26 from current levels of 3,666 beds (including recent sunshine acquisition) to 5,391 beds at average capex of Rs.8.14 Mn per bed.

Expansion plan for KIMS

Units	Tier	Allocation of capex (%)	Current Beds	Incremental Beds	Total	New Department	~ Capex (Rs Mn)	Approx timeline (In Months) From April 2021	Capex/ Bed (Rs Mn)
Kondapur	Tier I	24	200	500	700	All Specialities	3,000	36-42	6.0
Vizag	Tier II-III	16	434	50	484	Cancer Centre	150-200	24	3.5
Anantapur	Tier II-III	14	250	150	400	Cancer Centre/ Mother & Child	500-600	36-48	3.7
Ongole	Tier II-III	12	350	-	350	Cancer Centre	150-200	36-42	
Bangalore	Tier I	13	-	350-400	375	All Specialities	3,000-3,300	36	8.4
Chennai	Tier I	13	-	350-400	375	All Specialities	4,000	36	10.7
Western/ Central India	Tier I	9	-	250-300	275	All Specialities	3,000	24-36	10.9
Total				~1,650-1,800	2,959		~13,800-14,300		8.14

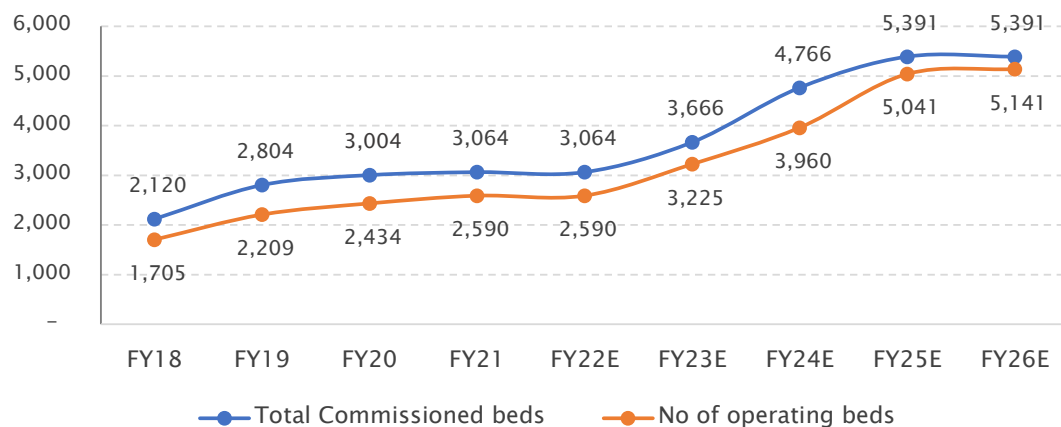
Source : Company, Dalal & Broacha research

Exhibit 18: We expect KIMS to touch topline of ~Rs.23,000 Mn by FY24 with sustained EBITDA of ~Rs.6,700 Mn



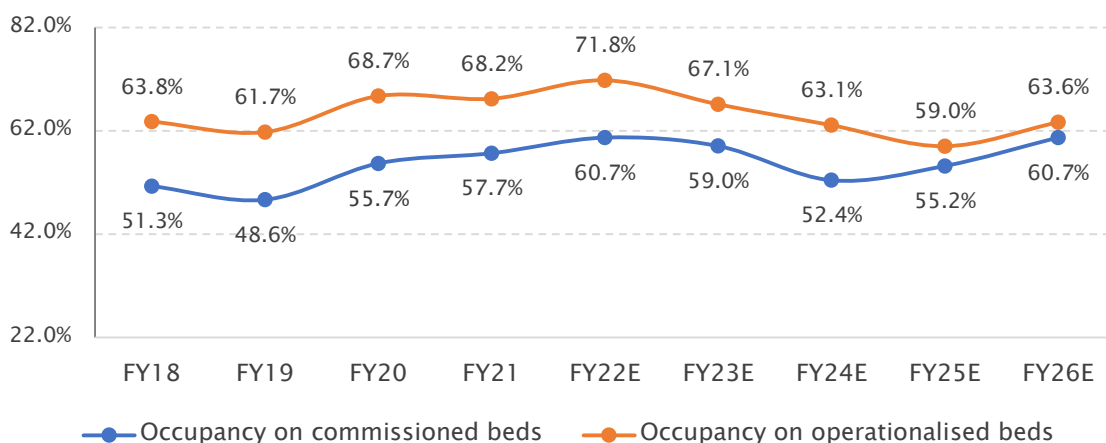
Source: Dalal & Broacha Research

Exhibit 19: We expect KIMS total commissioned beds to increase by ~50% from current levels of 3,064 beds to 5,391 beds by FY26 mainly through brownfield & greenfield expansion.



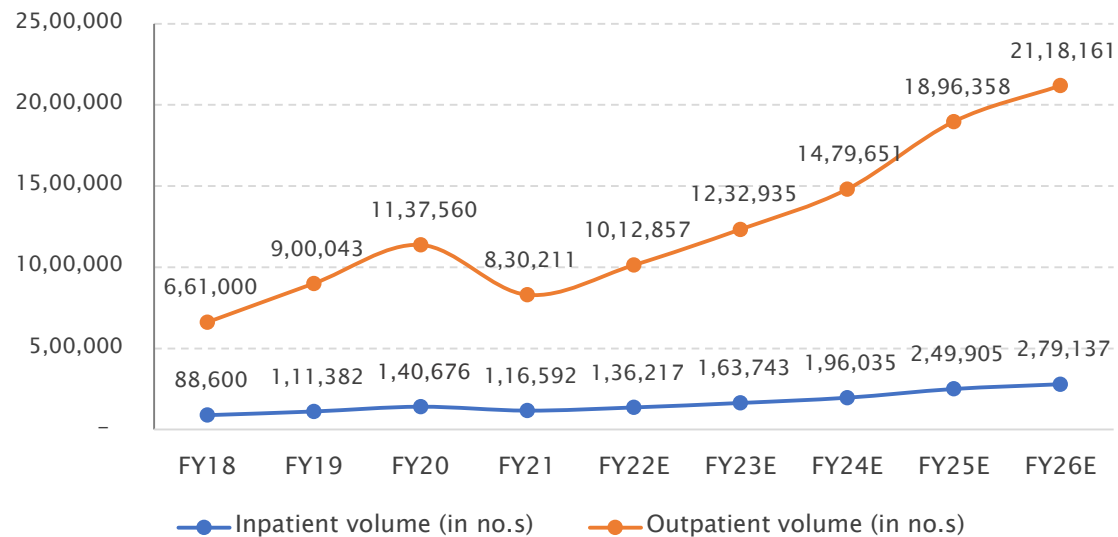
Source: Dalal & Broacha Research

Exhibit 20: We expect KIMS occupancy to rationalise in the range of 60-65% going forward as ramp up of newer facilities will take sometime to have company level occupancies.



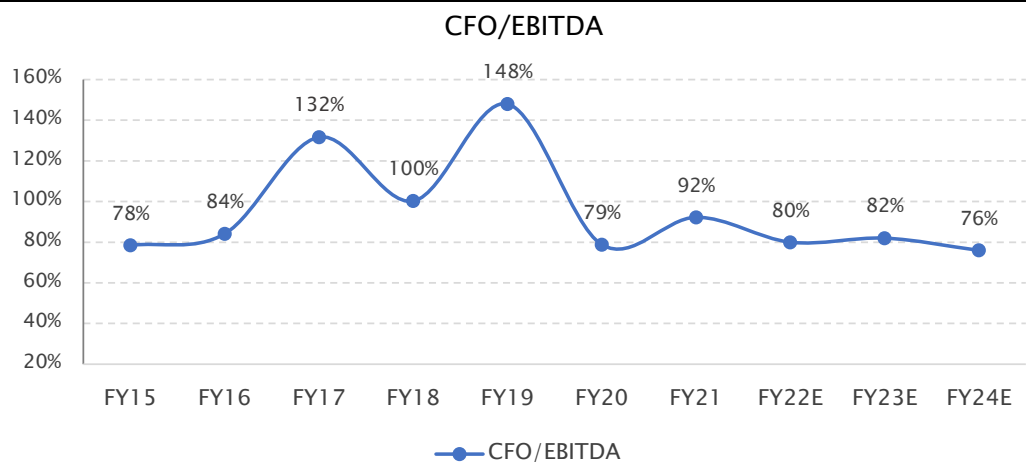
Source: Dalal & Broacha Research

Exhibit 21: We expect inpatient & out-patient volumes to grow at 20% CAGR between FY22-26 with ARPOB to grow at 5% CAGR



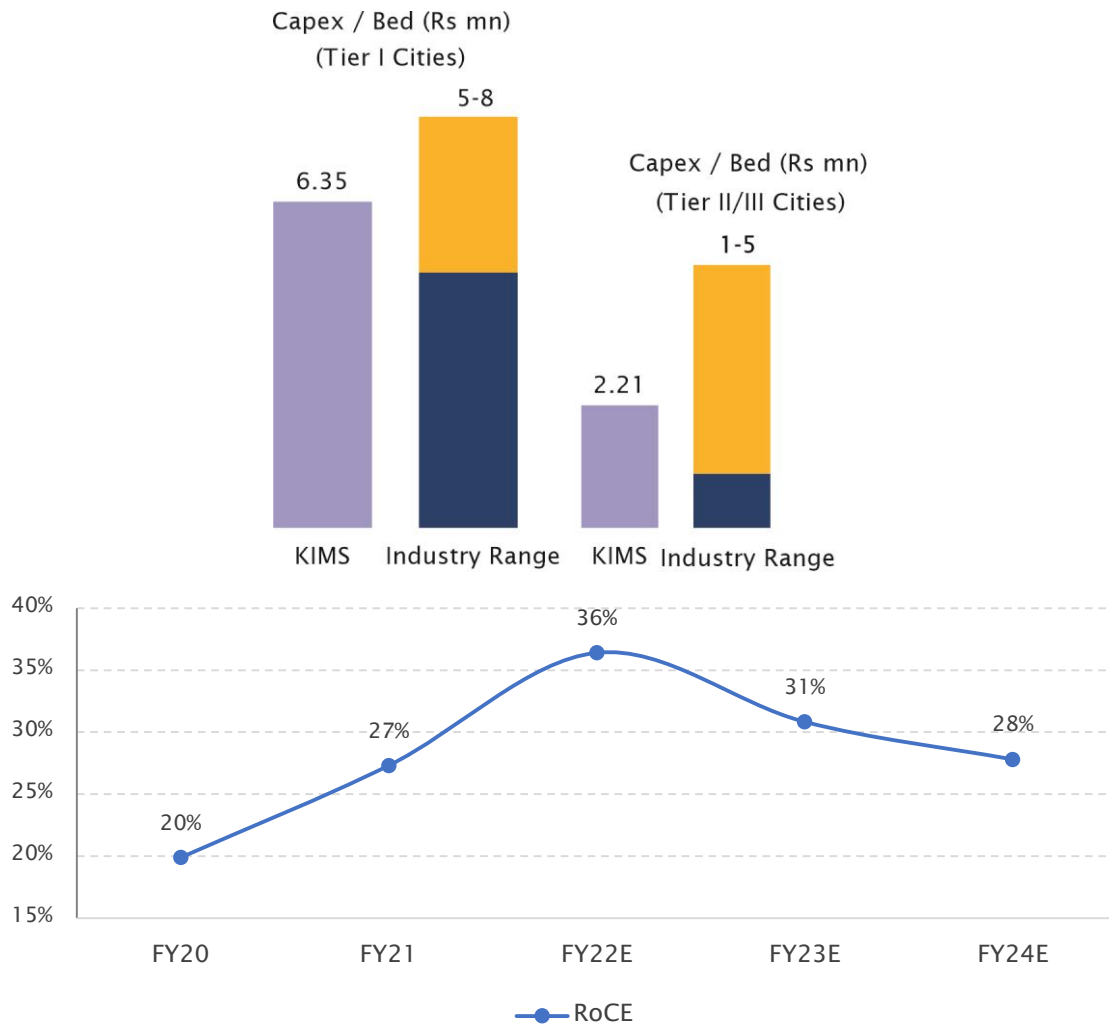
Source: Dalal & Broacha Research

Exhibit 22: KIMS has always seen healthy EBITDA to CFO conversion which in turn gets reinvested for generating new business either through acquisitions or green field projects. We expect KIMS to maintain CFO/EBITDA ratio in the range of 75-80%



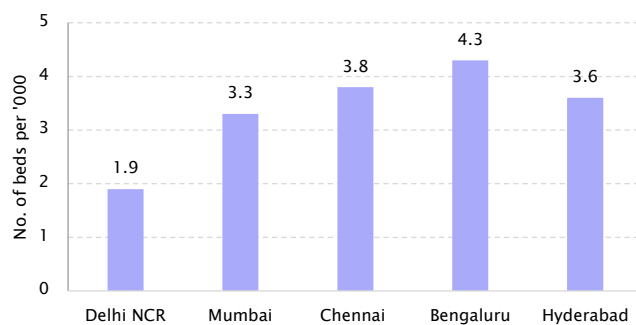
Source: Dalal & Broacha Research

Exhibit 23: KIMS ability to keep capex/bed low has allowed it to generate ROCE above 25%. We expect KIMS to maintain ROCE in the range of 20-25% despite huge capacity expansion in next 4-5 years.



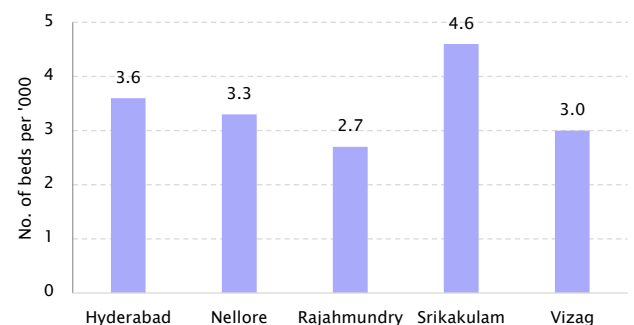
Source: Company Pre-IPO Presentation, Dalal & Broacha Research

Exhibit 24: Beds per '000 in metro cities are far lower than global average indicating under availability of healthcare facilities in India



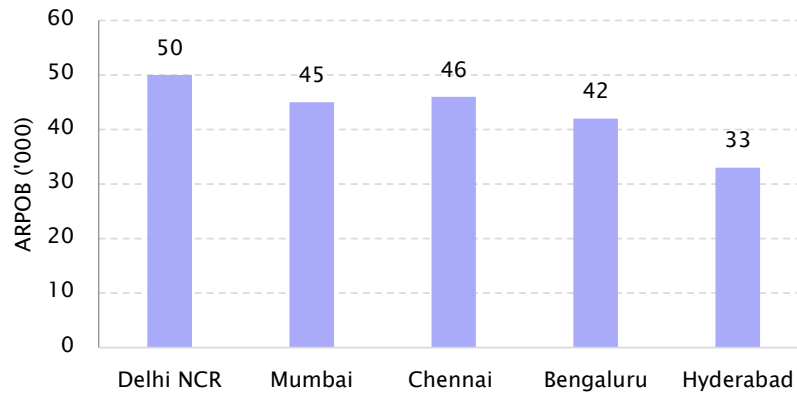
Source: Company, Dalal & Broacha research, Max Healthcare Investor Presentation

Exhibit 25: No. of Hospital Beds per '000 population across AP & Telangana. KIMS ability to tap underpenetrated tier II & III (Rajahmundry, Vizag) market creates a competitive edge over others whereas other large players are still struggling to enter



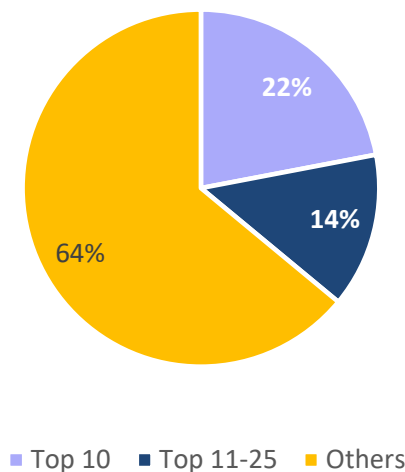
Source: Company, Dalal & Broacha research, RHP

Exhibit 26: Hyderabad has far lower ARPOB compared to other metro cities where KIMS has dominant presence indicating room for growth in ARPOB



Source: Company, Max Presentation, Dalal & Broacha Research

Exhibit 27: Top 25 doctors contributed 36% of overall revenue in FY21 which clearly indicates doctor participation model driving occupancies & thereby revenue.



Source: Company(FY21), Dalal & Broacha Research

Exhibit 28: KIMS ability to acquire sunshine at low cost (~9.5x EV/EBITDA which is far lower compared to listed peers trading at almost ~22-24 EV/EBITDA) will be value accretive for KIMS as sunshine is a well known brand in orthopedic & introduction on new specialities & cost rationalisation by KIMS will increase margins in next 2-3 years. There is significant room for EBITDA margin improvement incase of sunshine from current 18% to 25% in next 2-3 years.

Valuation	
Particulars	FY21
Contribution for 51.07% stake (₹ Mn.)	3,628
Total Enterprise Value for 100% stake (₹ Mn.)	7,104
Sunshine EBITDA (₹ Mn.)	752
Derived EV/EBITDA multiple (x)	9.45

Source: Company, Dalal & Broacha Research

Exhibit 29: KIMS Key operating metrics

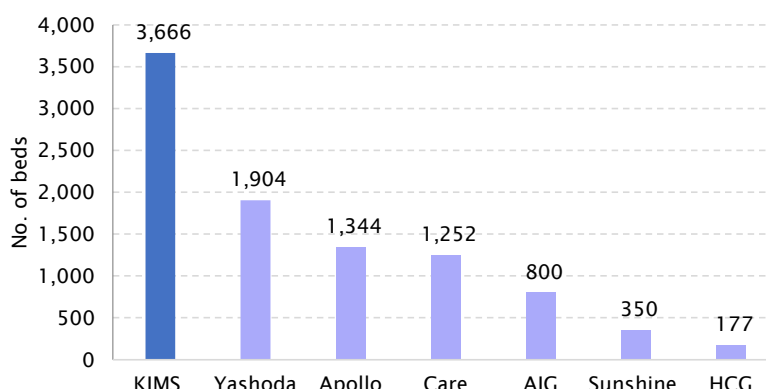
Particulars	FY18	FY19	FY20	FY21	FY22E	FY23E	FY24E	FY25E	FY26E	FY 22-26E CAGR
Total Commissioned beds (in no.)	2,120	2,804	3,004	3,064	3,064	3,666	4,766	5,391	5,391	
Additional beds (in no.s)		684	200	60	-	602	1,100	625	-	
Occupancy on commissioned beds(%)	51.3	48.6	55.7	57.7	60.7	59.0	52.4	55.2	60.7	
No of operating beds (in no.s)	1,705	2,209	2,434	2,590	2,590	3,225	3,960	5,041	5,141	
Occupancy on operationalised beds(%)	63.8	61.7	68.7	68.2	71.8	67.1	63.1	59.0	63.6	
No of occupied beds (in no.s)	1,087	1,364	1,673	1,766	1,859	2,165	2,498	2,975	3,271	
Inpatient volume (in no.s)	88,600	1,11,382	1,40,676	1,16,592	1,36,217	1,63,743	1,96,035	2,49,905	2,79,137	20
% Growth		26	26	-17	17	20	20	27	12	
In-patient revenues (₹ Mn)	5,947	7,177	8,799	10,481	13,405	15,801	18,132	24,187	27,151	19
% Growth		21	23	19	28	18	15	33	12	
Avg. length of stay (No.of days)	4.48	4.47	4.34	5.53	4.98	4.83	4.65	4.34	4.28	
Avg. revenue per occupied bed day (₹)	14,983	14,415	14,412	16,256	19,755	19,997	19,887	22,277	22,738	
% Growth		-4	0	13	22	1	-1	12	2	
Avg. revenue per Admission (₹)	67,122	64,436	62,548	89,895	98,409	96,497	92,493	96,784	97,267	
% Growth		-4	-3	44	9	-2	-4	5	0	
Outpatient Vol. (in no.s)	6,61,000	9,00,043	11,37,560	8,30,211	10,12,857	12,32,935	14,79,651	18,96,358	21,18,161	20
% Growth		36	26	-27	22	22	20	28	12	
Out-patient revenues (₹ Mn)	1,528	1,956	2,388	2,800	3,313	4,138	5,127	6,966	7,945	24
% Growth		28	22	17	18	25	24	36	14	
Avg revenue per registration (₹)	2,312	2,173	2,099	3,373	3,271	3,356	3,465	3,674	3,751	3
% Growth		-6	-3	61	-3	3	3	6	2	
Group Hospital revenues (₹ Mn)	7,475	9,133	11,187	13,281	16,718	19,938	23,259	31,153	35,096	20
% Growth		22	22	19	26	19	17	34	13	
ARPOB on total hospital revenues (₹ per day)	18,832	18,344	18,323	20,599	24,638	25,234	25,510	28,693	29,392	5
% Growth		-3	0	12	20	2	1	12	2	
ARPP (₹)	84,368	81,997	79,523	1,13,910	1,22,732	1,21,765	1,18,647	1,24,661	1,25,729	1
% Growth		-3	-3	43	8	-1	-3	5	1	

Source: Dalal & Broacha Research

Analysing Competition

KIMS has strong regional presence in southern India & scale leadership with bed capacity at ~1.9x that of second largest player in AP & Telangana.

Exhibit 30: No.of beds



Source: Company, Dalal & Broacha Research (includes recent sunshine acquisition)

Note : Yashoda will become largest player in AP & Telangana post commissioning of its facility in hitech city of 2,000 beds getting operational in ~1.5-2 years which is almost 20kms distance from KIMS flagship secunderabad facility.

Moat/Competitive Edge

KIMS' focus on underpenetrated tier2 & tier3 cities allowed it to create competitive edge where other large players are struggling due to high cost & lack of awareness about market mechanism. KIMS derives almost **36% of its total revenue & 26% of overall EBITDA in FY21** from tier 2 & tier 3 cities.

Patients in Kurnool, Anantapur & Vizag (Tier II & III Cities) lacked medical infrastructure that could perform complex therapies. They had to travel to nearest metro cities Bengaluru, Chennai or Hyderabad where majority of multi-speciality hospitals are present. In such situation their average cost of treatment would be generally higher because of higher cost of living in metro cities compared to Tier II & Tier III cities. Also they had to shell out extra on travel & accommodation in city.

KIMS saw this as an opportunity, with its brand presence in telugu speaking states, medical know-how, professional expertise, operational efficiency & execution capabilities it expanded or bought out hospitals in Tier II & Tier III cities & started complex therapies in those facilities creating a win-win situation for both KIMS & patients. Patients saved travel cost & average cost of treatment was also lower due to lower operational cost, inflation & rent in Tier II & Tier III cities compared to metros.

This gave KIMS a unique edge over other hospital players whose focus was more on metro cities with higher ARPOB. KIMS focused more on volumes (lower ARPOB but higher total revenue) & maintained 20-25% lower pricing than peers which attracted lot of patients to KIMS. Also the competition was lower in Tier II & Tier III cities as these complex therapies were a process of evolution & cannot be easily replicated by small hospitals.

Evolution process of a profitable hospital ; turnaround process

What drives margin in healthcare industry or evolution process of turning a hospital into profit powerhouse

For turning around a hospital, first ideal step is to **increase volume** (KIMS volume has grown 1.6x from FY18 to FY20 majorly through doctor participation model; FY21 ignored on account of covid) & **create brand awareness** (KIMS expanded its network through acquisitions in last 4 years in nearby regions thereby creating a strong regional brand).

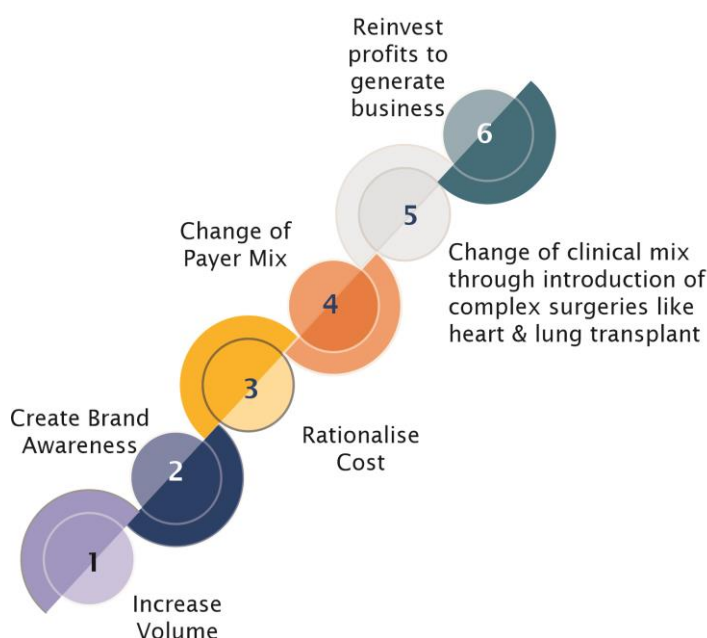
Doctor consultancy which has always been in range of ~20-22% of overall revenue & employee cost which has been in the range of ~15-18% of overall revenue cannot be compromised & adjusting them is very difficult as they grow proportionately with increase in beds. With increased volume one can increase the overall revenue & this in turn leads to operating leverage (reduced raw material cost & other expenses as a % to sales) thereby improving margins.

The next ideal step in turnaround process is to change the payer mix which further enhances margins (Corporates & aarogyasri payers as a % of overall payer mix has reduced which are low margin business; KIMS corporate & aarogyasri payer mix fell from 33% in FY21 to 21% to Q2FY22)

Post increase in volumes & changing payer mix, good doctors with rich experience tend to join the organisation which leads to introduction of complex surgeries thereby changing the overall clinical mix which further enhances the margins (introduction of complex surgeries like heart & lung transplant which was added in FY21 at flagship secunderabad facility). Reinvesting profits generated into right business by having high EBITDA to CFO conversion (which has been >80% incase of KIMS)

Exhibit 31: Margin expansion process in hospitals

Margin Expansion Process in Hospitals

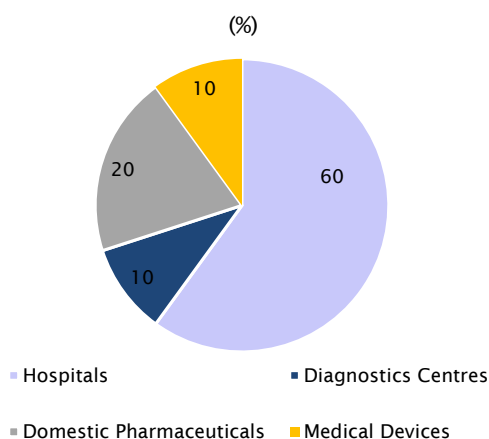


Source: Dalal & Broacha Research

Industry Analysis

The healthcare industry in India is poised for growth. The Indian healthcare delivery industry is expected to grow at a 17-18% CAGR (2020 - 2024E) and reach ₹ 7.07 trillion by 2024, according to CRISIL Report. In Fiscal Year 2020, 68% of hospital treatments, in terms of the treatment value, were carried out in private hospitals, and the number is expected to reach 72% in Fiscal Year 2024, according to the CRISIL Report. Out of the total healthcare spends, hospitals contribute ~60% of the overall pie followed by domestic pharmaceuticals at 20% & balance by diagnostic centres & medical devices.

Exhibit 32: Hospitals garner largest share of ~60% of overall healthcare spends in India & is expected to grow at 17-18% CAGR



Source: Crisil Report, RHP, Dalal & Broacha Research

Peer Comparison

Hospitals	CMP	No. of Shares (in Mn.)	M.Cap (in.Mn)	Revenue (in ₹ Mn.)				EBITDA (in ₹ Mn.)				Operating Profit Margin (%)			
				FY21	FY22E	FY23E	FY24E	FY21	FY22E	FY23E	FY24E	FY21	FY22E	FY23E	FY24E
Fortis	252	755	190,249	40,301	58,055	64,225	71,012	4,044	11,120	12,371	14,166	10%	19%	19%	20%
Max	343	970	332,569	36,010	52,364	55,678	63,348	6,080	13,341	14,754	17,197	17%	25%	26%	27%
Apollo	4,838	144	695,201	105,600	148,218	169,730	197,356	11,374	23,026	26,947	32,520	11%	16%	16%	16%
Narayana	680	204	138,965	25,823	36,569	41,367	46,799	1,821	6,215	7,474	8,779	7%	17%	18%	19%
Shalby Ltd	125	108	13,501	4,309	6,880	7,043	7,785	864	1,196	1,321	1,642	20%	17%	19%	21%
HCG	273	139	37,945	10,092	13,841	15,396	17,112	1,266	2,330	2,785	3,282	13%	17%	18%	19%
KIMS	1,330	80	106,437	13,299	16,718	19,938	23,259	3,709	5,270	5,839	6,727	28%	32%	29%	29%
V/s Consensus	1,330	80	106,437	13,299	16,729	22,552	24,410	3,709	5,293	6,269	6,897	28%	32%	28%	28%
Difference (in %)				0.00%	-0.06%	-11.59%	-4.72%	0.00%	-0.43%	-6.86%	-2.46%				

Hospitals	Adj.Net Profit (in ₹)				Net Profit Margin (%)				Adj.EPS (in ₹)				P/E				EV/EBITDA			
	FY21	FY22E	FY23E	FY24E	FY21	FY22E	FY23E	FY24E	FY21	FY22E	FY23E	FY24E	FY21	FY22E	FY23E	FY24E	FY21	FY22E	FY23E	FY24E
Fortis	(1,110)	3,810	5,283	6,708	-3%	7%	8%	9%	(1.5)	5.0	7.0	8.9	-172	50	36	28	-	19.6	17.6	15.4
Max	8,841	8,841	9,491	11,161	25%	17%	17%	18%	-	8.8	9.8	11.5	-	39	35	30	-	25.5	23.0	19.8
Apollo	1,504	9,997	12,183	16,181	1%	7%	7%	8%	10.7	65.0	83.0	110.9	450	74	58	44	29.2	31.1	26.6	22.0
Narayana	-	3,336	4,044	5,007	0%	9%	10%	11%	(0.7)	16.3	19.4	23.9	-971	42	35	28	-	21.7	18.0	15.4
Shalby Ltd	424	517	691	870	10%	8%	10%	11%	3.9	4.8	6.4	8.1	32	26	20	15	-	11.6	10.5	8.5
HCG	(1,277)	(11)	465	886	-13%	0%	3%	5%	(11.2)	(0.5)	3.5	6.7	-24	-576	77	41	-	18.6	15.6	13.2
KIMS	2,012	3,405	3,591	4,101	15%	20%	18%	18%	25.9	42.6	44.9	51.3	51	31	30	26	27.7	19.3	17.2	15.5
V/s Consensus	2,012	3,121	3,721	3,931	15%	19%	16%	16%	25.9	39.0	46.5	49.1	51	34	29	27	27.7	20.8	17.6	16.0
Difference (in %)									0.00%	9.14%	-3.47%	4.33%								

Source : Dalal & Broacha Research, Bloomberg

Key risks in the company

High dependency on healthcare professionals: The attrition rate for healthcare professionals, which includes resident doctors (including DNB students), consultant doctors, nursing staff (including interns) and paramedical personnel, for Fiscal Years 2019, 2020 and 2021 was 39.7%, 39.0% and 51.6%, respectively. The higher attrition rate in Fiscal Year 2021 was primarily due to higher attrition in nursing staff.

High dependency on one facility: KIMS derives 52%/63% of total revenue/EBITDA from Secunderabad facility. Any impact on the revenues from this hospitals could materially affect KIMS business, financial condition and cash flows.

Dependence on top doctors: KIMS derived ~22% revenue from top 10 doctors & ~36% revenue from top 25 doctors in FY21 indicating high dependency on few doctors.

Cessation of lease: Few hospitals of KIMS are on lease basis like Nellore hospital (30 years), Kodapur (10 years), Vizag, including recent sunshine acquisition & on termination of lease there are chances that lease agreement may not be renewed & can lead to cessation of operations & thereby affect revenue & profitability. Also KIMS Kondapur does not possess requisite occupancy certificate from the relevant municipal authority and fire NoC from Telangana State Disaster Response and Fire Services Department.

Clinical negligence : Brand is everything in hospital business. Any sort of clinical negligence can lead to adverse financial impact.

Government intervention ; In past there has been government interventions which has hampered the daily functioning of hospitals & impacted their revenue model.

Simplifying concepts

Key Concepts	
1	ARPOB : Average revenue per occupied bed ; it tells us how much revenue is generated by one occupied bed through inpatient volume
	ARPOB = Gross hospital revenue / occupied bed days (Note : Across industry there is a practice of taking gross hospital revenue as numerator instead of inpatient revenue for sake of uniformity)
2	ALOS : Average length of stay
	ALOS = Total length of stay / Inpatients volume (excluding dialysis and chemotherapy stays and volumes)
3	Occupied bed days ; it tells us how many days beds were occupied in a year
	Occupied bed days = Inpatient Volume * ALOS
4	Commissioned bed days ; it tells us how many bed days were available on commissioned bed basis
	Commissioned bed days = Commissioned beds * 365
5	ARPP : Average revenue per patient ; tells us how much revenue is generated per patient
	ARPP = ARPOB * ALOS
6	Average revenue per registration ; tells us average revenue generated by outpatient volume
	Average revenue per registration = Out-patient revenue / out-patient volume
7	Gross Hospital revenue
	Gross hospital revenue = Inpatient revenue + outpatient revenue
8	Aarogyasri Scheme :
	YSR Aarogyasri scheme is a flagship scheme to provide quality healthcare services to the poor which is implemented by the Andhra Pradesh & Telangana government
9	Tertiary Care
	Tertiary care require a higher level of specialty care with highly specialized equipment and expertise. Procedures such as coronary artery bypass surgery,dialysis,plastic surgeries,neurosurgeries,severe burn treatments etc.
10	Quaternary Care
	Quaternary care is considered an extension of tertiary care. However, it is even more specialized and highly unusual. Types of quaternary care include: Experimental medicine,procedures,Uncommon & specialized surgeries.

About the company & promoter background check

The company was incorporated as 'Jagjit Singh and Sons Private Limited', on July 26, 1973 at Mumbai. Until the year 2003, the company was owned, managed and controlled by Jagjit Singh and certain of his family members, who together owned the entire shareholding of the company. On February 15, 2003, acting in pursuance of the Takeover MoU, certain of their Promoters, namely Dr. Bhaskara Rao Bollineni and BRMH, along with certain other individuals and entities, acquired the entire equity share capital of the company (then 'Jagjit Singh and Sons Private Limited'). The company offers multidisciplinary healthcare services with primary, secondary, and tertiary care across 2-3 tier cities and an additional quaternary healthcare facility in tier-1 cities.

Dr. Bhaskara Rao Bollineni is the Promoter and Managing Director of the company. He has over 27 years of experience in cardiothoracic surgery and has in the past held various positions with Apollo Hospitals, Austin Hospital, University of Melbourne and Mahavir Hospital and Research Centre.

Anitha Dandamudi is the Whole-time Director of the company. She has over 16 years of experience in the hospital industry, having held various positions with the company, and has also served as vice president of administration at e-Talent Software Ltd.

Dr. Abhinay Bollineni is the Promoter and Executive Director of the company. He played a key role in establishing KIMS Kondapur in 2014.

Key Managerial Personnel

Vikas Maheshwari is the Chief Financial Officer of the company. He has been associated with the company since May 1, 2017. He has over 24 years of experience in accounting, finance and treasury.

Umashankar Mantha is the Company Secretary and General Manager (Legal) of the company. He has been associated with the company since July 1, 2015. He has over 17 years of experience in the secretarial and legal sectors.

FAQs

General FAQs	
1	How much time does it take to turnaround a hospital into profits?
	It usually takes 9-15 months to operationally breakeven for KIMS
2	What is KIMS doing differently?
	KIMS focuses on low cost affordable healthcare compared to peers which is the need of the hour, also its unique doctor participation model assures volume & hence better visibility in terms of volumes. 300 doctors own ~9% equity in the company. Also it plans to expand in nearby regions in Chennai, Bangalore & central India where the brand is prevalent as lot of people migrate to their hospitals from this region. It has robust capex plans which are explained in the report. It is spread across tier I, II & III cities whereas other all peers are concentrated in metro cities.
3	What is the reason of promoter pledge?
	Promoters run medical college with attached hospital which is a non profit organisation & not a part of KIMS listed entity for which pledged shares was given as a collateral to purchase land for the organisation. Also they had some personal level debt in family business done by promoters brothers & to secure loan for that business at lower rate shares have been pledged. Currently, (Q3FY22) the promoter pledge stands at 5% of overall equity.

4	Being in medical education help raise the talent pool for KIMS ?
	Management believes, it is an integral part of hospital business
5	Regional dependence of KIMS is very high, any way to mitigate or minimise that?
	Healthcare as such there is no risk with respect to regional dependency.Over the years with new hospitals coming on stream,its overall dependence of flagship secunderabad facility will reduce drastically which is explained in the report. Per capita income in South Indian region is far better compared to overall India.Within state of AP &Telangana company sees significant opportunity
6	In last 3 years EBITDA Margins have grown significantly,what would be stable set of margins
	Historically KIMS' has grown EBITDA at 20%+ CAGR (With blend of existing + new acquisition) & they expect revenue & EBITDA to grow in same range of 20% CAGR Matured Assets EBITDA stands at 31-32% ,Acquired Assets EBITDA stands at 17%
7	Top 25 doctors account for 36% of revenue, does it lead to risk for the company?
	High revenue contribution from top 25 doctors indicates that doctor participation model is working well & does not tantamount to risk.
8	Why does Kondapur facility have so high ARPOB of ~ Rs.45,000/- per bed
	Kondapur facility has better payer mix of cash & insurance business,& hardly there is any contribution from corporates or aarogyasri whereas other places have mixed payer mix
9	Are payments from government welfare schemes being fully realised?
	Yes,there might be delay in payment but amount is fully realised (last 18 months payment has been swift)
10	Is KIMS planning to get in the business of diagnostics or any related business?
	As of now KIMS has no plan for pharmacy or diagnostic business
11	What is the dividend policy KIMS intends to follow given that KIMS is cashflow positive company?
	Company aims to deploy free cashflow for growth capex going forward.They haven't decided on dividend policy
12	Out of top 25 doctors how many practice at KIMS & other hospitals?
	All of them are full time professionals in KIMS however they can work at their own clinics in evening
13	Do other states provide health schemes like Andhra Pradesh?
	No,other states don't have state health schemes,national schemes like ayushman bharat takes care
14	Why is the average stay at ~5 days despite robotic surgery & introduction of newer technologies?
	In state of Andhra Pradesh nearly 30% volume comes from state government schemes which usually have high ALOS & they have to apply one day in advance which also gets added to overall length of stay

P&L FAQs

15	In FY21 revenues increased despite no. of patients decreasing in FY21
	Focus on heart & lung transplant program led to increase ARPOBs & hence increase in overall revenue.
16	What led to increase of margins in FY21 ?
	Only 45 days were restricted from operations rest period KIMS was allowed to operate in the state of andhra pradesh which lead to increase in margin & there was no stoppage in elective surgeries during this period. However, Telangana state saw ups & downs on account of government restrictions. Thereby indicating steady revenue stream for the company
17	How has KIMS managed to outperform on operational ratios, what is it doing differently?
	KIMS Model : Large format hospitals, does not focus more on pricing but focuses on volumes, cater all sections of society. Operating leverage is extremely high due to high volumes.
18	What led to lower doctor cost in FY21?
	Acquired assets have fixed doctor pay, operating leverage kicked in due to higher volumes Nothing specifically was done on employee cost, so was largely similar to FY20

Sunshine (Subsidiary) FAQs

19	What led to margin expansion from 9% in FY2020 to 18% in FY2021 for sunshine?
	Growth in revenue : 40 crs Growth in EBITDA : 35 crs Due to operating leverage there was margin expansion Gachibowli newly opened project started turning around in FY21 Bhubaneswar was a 5-6 crs loss making entity in FY20 which was sold in FY21 so that also improved profitability Going forward KIMS targets an EBITDA of 24-26% from sunshine in 24-36 months Current margins are lower for sunshine as it has rented premises
20	What would be the expected occupancy for Sunshine?
	KIMS expects sunshine occupancy to go upto 45% vs historical 40%
21	What are the synergies with respect to Sunshine?
	Saving in terms of consumables to the tune of almost 10% of FY21 EBITDA of sunshine

Valuation & Outlook

Valuation	
Particulars	FY24
KIMS EBITDA (₹ Mn.)	6,727
Less : Sunshine EBITDA	966
KIMS EBITDA Ex of Sunshine	5,761
Target multiple (x)	20
Target EV - (A)	1,15,230
Sunshine EBITDA	966
KIMS Stake - 51%	492
Target multiple (x)	15
Target EV - (B)	7,386
Target EV (A + B)	1,22,616
Less : Debt	837
Add : Cash	3,084
Market Cap (₹ Mn.)	1,24,863
No.of O/s shares (Mn.)	80
Target price	1,561
CMP	1,330
Potential Upside (%)	17%

We initiate coverage on KIMS with “**BUY ON DIPS**” rating for a target price of Rs.1,561 valuing company at 20x EV/EBITDA of FY24E Ex-sunshine & 15x EV/EBITDA of FY24E for sunshine.

KIMS has delivered healthy revenue/EBITDA/PAT growth of 21%/28%/49% between FY16-21. Going forward we expect KIMS to deliver revenue/EBITDA/PAT growth of 20%/19%/17% between FY22-26.

We believe KIMS’ ability to rationalize cost & maintain industry leading margins along with affordable pricing is a perfect blend of leader in the making. Management has guided for ~20% revenue growth for next few years along with sustained EBITDA margin profile of 30% along with ARPOB in the range of Rs.24,000-25,000. With its regional dominance & upcoming new brownfield & greenfield expansion of Rs.14,000 mn KIMS is all set for expansion spree backed by robust EBITDA to operating cashflow conversion of >80%. Also its unique business model of doctor participation brings visibility in terms of volume. We believe investors should buy the stock on dips for a **target price of Rs.1,561**

Financials

P&L (Rs mn)	FY20	FY21	FY22E	FY23E	FY24E	Cash Flow St. (Rs. mn)	FY20	FY21	FY22E	FY23E	FY24E
Net Sales	11,226	13,299	16,718	19,938	23,259	Net Profit	1,192.3	2,011.9	3,405.1	3,591.1	4,100.5
Raw Material Cost	(2,542)	(2,889)	(3,678)	(4,386)	(5,117)	Add: Dep. & Amort.	706.1	695.4	695.4	853.4	1,001.9
Employee Cost	(1,980)	(2,202)	(2,528)	(3,127)	(3,747)	Cash profits	1,898.4	2,707.2	4,100.5	4,444.5	5,102.4
Other Expenses	(4,254)	(4,499)	(5,242)	(6,586)	(7,668)	(Inc)/Dec in					
Operating Profit (EBITDA)	2,450	3,709	5,270	5,839	6,727	Sundry debtors	(90.0)	224.5	(184.3)	(83.1)	(227.5)
Depreciation	(706)	(695)	(695)	(853)	(1,002)	Inventories	(35.2)	62.9	(79.8)	(61.8)	(63.7)
PBIT	1,744	3,013	4,575	4,985	5,725	Loans/advances	(9.7)	(121.6)	-	-	-
Other income	61	102	112	123	135	Other Current Assets	290.9	(227.8)	-	-	-
Interest	(399)	(325)	(50)	(51)	(84)	Current Liab and Provisions	95.9	58.8	52.6	245.3	58.0
PBT	1,405	2,790	4,637	5,057	5,777	Sundry Creditors	193.8	84.4	293.6	250.4	240.2
Extraordinary Fixed term consultant fees	-	-	-	-	-	Change in working capital	445.7	81.1	82.1	350.8	7.0
Profit before tax (post exceptional)	1,405	2,790	4,637	5,057	5,777	CF from Oper. activities	2,344.1	2,788.4	4,182.6	4,795.3	5,109.4
Provision for tax	(255)	(735)	(1,167)	(1,273)	(1,454)	CF from Inv. activities	(1,233.9)	(916.5)	-	(7,224.6)	(9,041.2)
Reported PAT	1,151	2,055	3,470	3,784	4,323	CF from Fin. activities	(634.2)	523.9	(1,810.1)	3,680.8	546.9
MI	(42)	43	64	193	222	Cash generated/(utilised)	476.0	2,395.8	2,372.5	1,251.5	(3,384.9)
Net Profit	1,192	2,012	3,405	3,591	4,101	Cash at start of the year	(27.5)	448.8	2,844.5	5,217.0	6,468.4
Adjusted Profit (excl Exceptionals)	1,192	2,012	3,405	3,591	4,101	Cash at end of the year	448.5	2,844.6	5,217.0	6,468.4	3,083.6
Balance Sheet	FY20	FY21	FY22E	FY23E	FY24E	Ratios	FY20	FY21	FY22E	FY23E	FY24E
Equity capital	745	776	800	800	800	OPM	21.8	27.9	31.5	29.3	28.9
CCPS	-	-	-	-	-	NPM	10.56	15.01	20.23	17.90	17.53
Reserves	5,236	7,861	11,267	14,858	18,958	Tax rate	(18.1)	(26.4)	(25.2)	(25.2)	(25.2)
Net worth	5,981	8,637	12,067	15,658	19,758	Growth Ratios (%)					
MI	125	125	189	3,858	4,080	Net Sales	22.3	18.5	25.7	19.3	16.7
Non Current Liabilities	3,656	2,816	1,244	1,252	1,562	Operating Profit	202.6	51.4	42.1	10.8	15.2
Current Liabilities	2,188	2,783	2,804	3,302	3,616	PBIT	611.7	72.8	51.8	9.0	14.8
TOTAL LIABILITIES	11,950	14,362	16,303	24,070	29,016	PAT	(349.4)	68.7	69.3	5.5	14.2
Non Current Assets	9,761	9,850	9,154	15,525	23,565	Per Share (Rs.)					
Fixed Assets	8,334	8,555	7,860	9,528	17,567	Net Earnings (EPS)	16.01	25.93	42.56	44.89	51.26
Goodwill	848	848	848	5,551	5,551	Cash Earnings (CPS)	25.5	34.9	51.3	55.6	63.8
Non Current Investments	-	-	-	-	-	Dividend	-	-	-	-	-
Deferred Tax Asset	14	29	29	29	29	Book Value	80.3	111.3	150.8	195.7	247.0
Advances	47	164	164	164	164	Free Cash Flow	22.0	29.3	52.6	29.5	(45.1)
Other Non Current Assets	517	253	253	253	253	Valuation Ratios					
Current Assets	2,190	4,512	7,149	8,545	5,451	P/E(x)	83.1	51.3	31.2	29.6	25.9
Current investments	-	-	-	-	-	P/B(x)	16.6	11.9	8.8	6.8	5.4
Inventories	304	241	321	382	446	EV/EBIDTA(x)	41.4	27.7	19.3	17.2	15.5
Trade Receivables	1,323	1,098	1,282	1,366	1,593	Div. Yield(%)	-	-	-	-	-
Cash and Bank Balances	449	2,844	5,217	6,468	3,084	FCF Yield(%)	1.7	2.2	4.0	2.2	(3.4)
Advances	18	23	23	23	23	Return Ratios (%)					
Other Current Assets	97	306	306	306	306	ROE	20%	23%	28%	23%	21%
TOTAL ASSETS	11,950	14,362	16,303	24,070	29,016	ROCE	20%	27%	36%	31%	28%

Source: Company,Dalal & Broacha Research

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